

# Nursing Management Smart Bank

Complete corrected question bank

MCQ, True/False, Matching, Essay

# Chapter 1 - Urinary System Disorders

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## TRUE / FALSE

Ch 1 | #1 | TRUE / FALSE

**A urinary tract infection (UTI) can occur in the kidneys, ureters, bladder, or urethra.**

**Answer: T**

Ch 1 | #2 | TRUE / FALSE

**Women get UTIs less frequently than men because their urethra is longer.**

**Answer: F**

Ch 1 | #3 | TRUE / FALSE

**Elderly people in nursing homes are at higher risk for UTIs.**

**Answer: T**

Ch 1 | #4 | TRUE / FALSE

**Pregnancy is NOT a risk factor for developing a UTI.**

**Answer: F**

Ch 1 | #5 | TRUE / FALSE

**Immobility is a risk factor for urinary tract infection.**

**Answer: T**

Ch 1 | #6 | TRUE / FALSE

**Hematuria (blood in the urine) is a symptom of UTI.**

**Answer: T**

Ch 1 | #7 | TRUE / FALSE

**Nocturia refers to the absence of urination at night.**

**Answer: F**

Ch 1 | #8 | TRUE / FALSE

**A urine culture is performed to identify the type of bacteria and choose the appropriate antibiotic.**

**Answer: T**

Ch 1 | #9 | TRUE / FALSE

**A mild case of cystitis always requires antibiotic treatment.**

**Answer: F**

Ch 1 | #10 | TRUE / FALSE

**Chronic or recurrent UTI may require antibiotic therapy for up to 6 months to 2 years.**

**Answer: T**

Ch 1 | #11 | TRUE / FALSE

**Patients should wipe from back to front after toileting to prevent UTIs.**

**Answer: F**

Ch 1 | #12 | TRUE / FALSE

**Urinating after sexual intercourse helps prevent UTIs.**

**Answer: T**

Ch 1 | #13 | TRUE / FALSE

**Alcohol and caffeine are recommended fluids for patients with UTI.**

**Answer: F**

Ch 1 | #14 | TRUE / FALSE

**Renal failure is defined as total or nearly total loss of kidney function.**

**Answer: T**

Ch 1 | #15 | TRUE / FALSE

**There is only one type of renal failure.**

**Answer: F**

Ch 1 | #16 | TRUE / FALSE

**Diabetic nephropathy is the most common/associated/chronic renal failure.**

**Answer: T**

Ch 1 | #17 | TRUE / FALSE

**Hypertension nephrosclerosis is the leading cause of chronic renal failure.**

**Answer: F**

Ch 1 | #18 | TRUE / FALSE

**Neurogenic bladder is caused by impaired nerve fibers from the bladder to the spinal cord.**

**Answer: T**

Ch 1 | #19 | TRUE / FALSE

**Bladder outlet obstruction due to an enlarged prostate gland is a post-renal cause of CRF.**

**Answer: T**

Ch 1 | #20 | TRUE / FALSE

**Neurogenic bladder is caused by impaired nerve fibers from the bladder to the spinal cord.**

**Answer: T**

Ch 1 | #21 | TRUE / FALSE

**Poor cardiac function is a renal cause of chronic renal failure.**

**Answer: F**

Ch 1 | #22 | TRUE / FALSE

**Kussmaul breathing in CRF patients is caused by metabolic alkalosis.**

**Answer: F**

Ch 1 | #23 | TRUE / FALSE

**Pruritus (itching) in CRF patients is due to accumulation of urea frost on the skin.**

**Answer: T**

Ch 1 | #24 | TRUE / FALSE

**Salt and water retention in CRF can lead to congestive heart failure and hypertension.**

**Answer: T**

Ch 1 | #25 | TRUE / FALSE

**Hemodialysis works by circulating blood through a semi-permeable membrane (dialyzer) to remove toxins.**

**Answer: T**

Ch 1 | #26 | TRUE / FALSE

**An AV fistula involves the surgical joining of an artery and vein under the skin.**

**Answer: T**

Ch 1 | #27 | TRUE / FALSE

**Kidney stones in both ureters are a pre-renal cause of CRF.****Answer: F**

Ch 1 | #28 | TRUE / FALSE

**Renal biopsy is one of the laboratory/diagnostic studies used to evaluate CRF.****Answer: T**

Ch 1 | #29 | TRUE / FALSE

**Osteomalacia and renal osteodystrophy are skeletal complications of chronic renal failure.****Answer: T**

Ch 1 | #30 | TRUE / FALSE

**Dialysis is only indicated when a kidney transplant is not possible.****Answer: F**

## MCQ

Ch 1 | #31 | MCQ

**A UTI is best defined as:**

- A. Inflammation of the kidney parenchyma only
- B. An infection anywhere along the urinary tract including kidneys, ureters, bladder, or urethra
- C. A bacterial infection limited to the urethra
- D. A viral infection of the bladder only

**Answer: B**

UTI may involve any part of the urinary tract.

Ch 1 | #32 | MCQ

**Which anatomical reason makes women more prone to UTIs than men?**

- A. Women have a wider urethra
- B. Women have a longer urethra closer to the vagina
- C. Women have a shorter urethra closer to the anus
- D. Women produce less urine

**Answer: C**

Shorter female urethra and proximity to the anus increase bacterial access.

Ch 1 | #33 | MCQ

**Which of the following is NOT listed as a risk factor for UTI?**

- A. Pregnancy
- B. Kidney stones
- C. Regular physical exercise
- D. Catheterization

**Answer: C**

Regular physical exercise is not listed as a UTI risk factor.

Ch 1 | #34 | MCQ

**Cloudy urine and foul odor are symptoms of:**

- A. Diabetes mellitus
- B. Urinary tract infection
- C. Chronic renal failure
- D. Renal calculi

**Answer: B**

Cloudy, foul-smelling urine is a common UTI symptom.

Ch 1 | #35 | MCQ

**The diagnostic test that identifies the specific bacteria causing a UTI and guides antibiotic choice is:**

- A. Complete blood count
- B. Urinalysis
- C. Urine culture
- D. Renal biopsy

**Answer: C**

identifies organism, guides antibiotics

Ch 1 | #36 | MCQ

**Urinalysis in a patient with UTI typically reveals:**

- A. Glucose and ketones
- B. White blood cells or red blood cells
- C. Elevated protein only
- D. Normal findings

**Answer: B**

typical UTI urinalysis finding

Ch 1 | #37 | MCQ

**A patient with chronic or recurrent UTI may require antibiotic therapy for:**

- A. 3 to 5 days
- B. 1 to 3 weeks
- C. 1 to 3 months
- D. 6 months to 2 years

**Answer: D**

prolonged therapy for recurrence

Ch 1 | #38 | MCQ

**Which instruction is MOST important to prevent UTIs in a female patient?**

- A. Wipe from back to front
- B. Avoid drinking water
- C. Wipe from front to back
- D. Use synthetic undergarments

**Answer: C**

reduces fecal contamination

Ch 1 | #39 | MCQ

**Which of the following fluids should be AVOIDED by a UTI patient?**

- A. Water
- B. Cranberry juice
- C. Herbal tea
- D. Alcohol

**Answer: D**

bladder irritant to avoid

Ch 1 | #40 | MCQ

**Urinating after sexual intercourse is recommended because it helps:**

- A. Prevent pregnancy
- B. Flush bacteria from the urethra
- C. Reduce bladder spasms
- D. Decrease urine production

**Answer: B**

flushes bacteria out

Ch 1 | #41 | MCQ

**Surgery for UTI is recommended when:**

- A. The patient has recurrent UTIs
- B. The patient is elderly
- C. An anatomical abnormality is present causing the infection
- D. Antibiotic therapy has lasted 1 week

**Answer: C**

corrects structural cause

Ch 1 | #42 | MCQ

**Wearing cloth (cotton) undergarments is recommended to prevent UTIs because:**

- A. Cotton is more absorbent of urine
- B. Cotton allows better air circulation, reducing moisture and bacterial growth
- C. Synthetic materials are more comfortable
- D. Cotton kills bacteria directly

**Answer: B**

keeps area drier

Ch 1 | #43 | MCQ

**Renal failure is defined as:**

- A. Temporary reduction in urine output
- B. Inflammation of the kidney
- C. Total or nearly total loss of kidney function
- D. Obstruction of the ureter

**Answer: C**

near-total kidney loss

Ch 1 | #44 | MCQ

**How many types of renal failure are described?**

- A. One
- B. Two
- C. Three
- D. Four

**Answer: B**

acute and chronic



Ch 1 | #45 | MCQ

**Which classification includes poor cardiac function as a cause of CRF?**

- A. Renal causes
- B. Post-renal causes
- C. Pre-renal causes
- D. Intrinsic renal causes

**Answer: C**

reduced renal perfusion

Ch 1 | #46 | MCQ

**Neurogenic bladder causing CRF is classified as:**

- A. Pre-renal cause
- B. Post-renal cause
- C. Intrinsic renal cause
- D. Infectious cause

**Answer: B**

obstructive/post-renal problem

Ch 1 | #47 | MCQ

**The MOST common cause of chronic renal failure is:**

- A. Hypertension Nephrosclerosis
- B. Polycystic kidney disease
- C. Diabetic nephropathy
- D. Chronic glomerulonephritis

**Answer: C**

commonest CRF cause

Ch 1 | #48 | MCQ

**The SECOND leading cause of chronic renal failure is:**

- A. Polycystic kidney disease
- B. Diabetic nephropathy
- C. Hypertension Nephrosclerosis
- D. Chronic pyelonephritis

**Answer: C**

second leading cause

Ch 1 | #49 | MCQ

**Polycystic kidney disease is characterized by:**

- A. Inflammation of the glomeruli
- B. Multiple cysts in the kidneys
- C. Scarring of the renal cortex
- D. Enlarged renal pelvis

**Answer: B**

hallmark of PKD

Ch 1 | #50 | MCQ

**Which gastrointestinal symptom is associated with chronic renal failure?**

- A. Melena and constipation
- B. Anorexia, nausea, vomiting, and diarrhea
- C. Jaundice and hepatomegaly
- D. Dysphagia and heartburn

**Answer: B**

classic GI manifestations

Ch 1 | #51 | MCQ

**Rapid deep breathing (Kussmaul breathing) in CRF patients is caused by:**

- A. Pulmonary edema
- B. Metabolic alkalosis
- C. Metabolic acidosis
- D. Respiratory infection

**Answer: C**

acid retention causes Kussmaul

Ch 1 | #52 | MCQ

**Pruritus (generalised itching) in CRF patients is caused by:**

- A. Liver failure
- B. Allergic reaction to medications
- C. Accumulation of urea frost on the skin
- D. Calcium deposits in the skin

**Answer: C**

urea frost irritation

Ch 1 | #53 | MCQ

**Which skin change is associated with CRF?**

- A. Pale white skin
- B. Red flushed skin
- C. Yellow or brown skin discolouration with increased pigmentation
- D. Blue cyanotic skin

**Answer: C**

typical CRF pigmentation change

Ch 1 | #54 | MCQ

**Salt and water retention in CRF leads to all of the following EXCEPT:**

- A. Edema
- B. Congestive heart failure
- C. Hypertension
- D. Hypotension

**Answer: D**

retention does not cause hypotension

Ch 1 | #55 | MCQ

**Neurological manifestations of CRF include all of the following EXCEPT:**

- A. Confusion and drowsiness
- B. Muscle cramps and seizures
- C. Coma
- D. Improved mental alertness

**Answer: D**

alertness does not improve

Ch 1 | #56 | MCQ

**Which laboratory panel is used in the diagnostic evaluation of CRF?**

- A. Liver function tests only
- B. Full electrolyte panel including calcium, phosphate, uric acid, magnesium, and albumin
- C. Thyroid function tests
- D. Coagulation studies only

**Answer: B**

broad metabolic assessment

Ch 1 | #57 | MCQ

**Which radiographic study evaluates for obstruction, stones, tumour, and kidney size in CRF?**

- A. MRI
- B. Angiography
- C. Renal ultrasound
- D. Chest X-ray

**Answer: C**

checks size/obstruction/stones

Ch 1 | #58 | MCQ

**The goal of conservative management in CRF is:**

- A. Complete cure of the disease
- B. Retaining kidney function as much as possible
- C. Immediate preparation for transplant
- D. Replacing kidney function entirely

**Answer: B**

preserve remaining function

Ch 1 | #59 | MCQ

**Which medication may be used to treat anemia in CRF patients?**

- A. Insulin injections
- B. Iron and erythropoietin injections
- C. Calcium supplements only
- D. Diuretics

**Answer: B**

treats renal anemia

Ch 1 | #60 | MCQ

**Dialysis is a process that:**

- A. Cures chronic renal failure permanently
- B. Replaces only hormone functions of the kidney
- C. Removes fluid, waste products, and toxic substances from blood when kidneys cannot work
- D. Increases urine production in failing kidneys

**Answer: C**

removes waste and fluid

Ch 1 | #61 | MCQ

**In hemodialysis, blood is circulated through:**

- A. The peritoneal cavity
- B. A semi-permeable membrane (dialyzer)
- C. A synthetic heart valve
- D. A venous reservoir only

**Answer: B**

blood passes dialyzer

Ch 1 | #62 | MCQ

**Vascular access for hemodialysis can be achieved through:**

- A. A central venous catheter only
- B. An AV fistula or AV graft
- C. A peripheral IV line
- D. A Foley catheter

**Answer: B**

standard hemodialysis access

Ch 1 | #63 | MCQ

**An AV fistula is created by:**

- A. Inserting a synthetic tube between artery and vein
- B. Surgically joining an artery and vein under the skin
- C. Cannulating the femoral artery
- D. Placing a catheter in the subclavian vein

**Answer: B**

artery-vein surgical connection

Ch 1 | #64 | MCQ

**The two replacement therapy options for lost kidney function are:**

- A. Hemodialysis and peritoneal dialysis only
- B. Transplant and dialysis
- C. Medications and dietary changes
- D. Surgery and radiation

**Answer: B**

main replacement options

Ch 1 | #65 | MCQ

**Atherosclerosis of the renal arteries causes CRF by:**

- A. Blocking urine outflow from the kidney
- B. Directly destroying nephrons
- C. Inducing ischaemic nephropathy through reduced blood flow
- D. Causing polycystic changes

**Answer: C**

ischemia from low flow

Ch 1 | #66 | MCQ

**Bone disease in CRF (renal osteodystrophy) is caused by:**

- A. Excessive calcium absorption
- B. Impaired calcium and phosphate metabolism due to kidney failure
- C. Vitamin B12 deficiency
- D. Steroid medication use only

**Answer: B**

disordered Ca/phosphate balance

Ch 1 | #67 | MCQ

**Which of the following correctly describes pressure in the lower pelvis as a symptom?**

- A. It is specific to chronic renal failure
- B. It is a symptom of UTI
- C. It only occurs in men with prostate disease
- D. It is a neurological symptom of CRF

**Answer: B**

common UTI symptom

Ch 1 | #68 | MCQ

**Bowel incontinence is a risk factor for UTI because:**

- A. It reduces fluid intake
- B. Faecal contamination increases bacterial access to the urethra
- C. It causes bladder stones
- D. It affects kidney function directly

**Answer: B**

fecal bacteria spread

Ch 1 | #69 | MCQ

**Frequent and urgent need to urinate as a symptom of UTI is also called:**

- A. Oliguria
- B. Dysuria
- C. Urinary urgency/frequency
- D. Anuria

**Answer: C**

urgency and frequency

Ch 1 | #70 | MCQ

**Which of the following is a pre-renal cause of chronic renal failure?**

- A. Polycystic kidney disease
- B. Bladder stone causing outlet obstruction
- C. Chronic liver failure reducing renal perfusion
- D. Kidney stones in both ureters

**Answer: C**

lowers renal perfusion

Ch 1 | #71 | MCQ

**Keeping the genital area clean is a preventive measure for:**

- A. Chronic renal failure
- B. Urinary tract infection
- C. Kidney stones
- D. Neurogenic bladder

**Answer: B**

prevents UTI

Ch 1 | #72 | MCQ

**Which finding on urinalysis indicates possible UTI?**

- A. Presence of glucose
- B. Presence of white blood cells (WBCs)
- C. Normal specific gravity
- D. Absence of protein

**Answer: B**

WBCs suggest infection

Ch 1 | #73 | MCQ

**Decreased fluid intake is a risk factor for UTI because:**

- A. It increases urine concentration, killing bacteria
- B. It causes urinary stasis, allowing bacteria to multiply
- C. It reduces kidney function directly
- D. It increases bladder capacity

**Answer: B**

stasis promotes bacterial growth

Ch 1 | #74 | MCQ

**A patient with CRF has fatigue, headache, and a general feeling of illness. This is called:**

- A. Uremia
- B. Azotemia
- C. General ill feeling (uremic syndrome)
- D. Nephrotic syndrome

**Answer: C**

uremic general malaise

Ch 1 | #75 | MCQ

**Angiography or CT scan in CRF patients is used to evaluate:**

- A. Bladder capacity
- B. Renal artery stenosis
- C. Urinary flow rate
- D. Electrolyte levels

**Answer: B**

evaluates renal artery narrowing

Ch 1 | #76 | MCQ

**In CRF management, which associated diseases must be controlled first?**

- A. Diabetes only
- B. Hypertension, congestive heart failure, UTIs, and kidney stones
- C. Anemia and osteoporosis only
- D. Neurological disorders

**Answer: B**

comorbid causes must be controlled



Ch 1 | #77 | MCQ

**Decreased sensation in hands, feet, or other areas in CRF is a manifestation of:**

- A. Cardiovascular complications
- B. Neurological manifestations (peripheral neuropathy)
- C. Skin changes
- D. Gastrointestinal complications

**Answer: B**

peripheral neuropathy sign

Ch 1 | #78 | MCQ

**The purpose of erythropoietin injection in CRF management is to:**

- A. Lower blood pressure
- B. Stimulate red blood cell production and control anemia
- C. Reduce potassium levels
- D. Treat metabolic acidosis

**Answer: B**

increases RBC production

Ch 1 | #79 | MCQ

**Which preventive measure helps reduce bacterial growth by maintaining a dry perineal environment?**

- A. Using synthetic underwear
- B. Wearing tight-fitting clothing
- C. Wearing cloth (cotton) undergarments
- D. Using scented soaps

**Answer: C**

reduces moisture, bacterial growth

Ch 1 | #80 | MCQ

**Coma in a patient with CRF is classified as a:**

- A. Cardiovascular complication
- B. Neurological manifestation
- C. Dermatological change
- D. Gastrointestinal symptom

**Answer: B**

CNS complication

Ch 1 | #81 | MCQ

**Which of the following correctly states the relationship between diabetes and CRF?**

- A. Diabetes never causes kidney disease
- B. Diabetic nephropathy is the most common cause of CRF
- C. Diabetes causes only pre-renal failure
- D. CRF causes diabetes mellitus

**Answer: B**

diabetes commonly leads to CRF

Ch 1 | #82 | MCQ

**Ascites in CRF patients results from:**

- A. Liver cirrhosis only
- B. Salt and water retention causing fluid accumulation in the peritoneum
- C. Excessive protein intake
- D. Calcium deposits in the abdomen

**Answer: B**

fluid accumulates from retention

Ch 1 | #83 | MCQ

**Nail abnormalities in CRF patients are classified as:**

- A. Neurological manifestations
- B. Gastrointestinal symptoms
- C. Skin and nail changes
- D. Cardiovascular complications

**Answer: C**

integumentary/nail change

Ch 1 | #84 | MCQ

**The purpose of renal ultrasound in CRF diagnosis is to:**

- A. Measure glomerular filtration rate
- B. Evaluate for obstruction, stones, tumour, kidney size, and chronic change
- C. Determine urine culture results
- D. Assess renal blood flow only

**Answer: B**

evaluates chronic structural changes

Ch 1 | #85 | MCQ

**Which of the following is a correct prevention strategy for UTIs?**

- A. Drink plenty of fluids to flush bacteria
- B. Reduce fluid intake to rest the bladder
- C. Use synthetic undergarments for better protection
- D. Urinate less frequently to retain bladder capacity

**Answer: A**

helps flush bacteria

Ch 1 | #86 | MCQ

**Hiccups in a patient with CRF are classified as:**

- A. A respiratory symptom
- B. A gastrointestinal manifestation of CRF
- C. A neurological sign
- D. A cardiovascular complication

**Answer: B**

GI manifestation

Ch 1 | #87 | MCQ

**Pain or burning with urination is medically termed:**

- A. Nocturia
- B. Hematuria
- C. Dysuria
- D. Proteinuria

**Answer: C**

painful urination term

Ch 1 | #88 | MCQ

**The process by which blood is cleaned in hemodialysis uses the principle of:**

- A. Osmosis only
- B. Diffusion across a semi-permeable membrane
- C. Filtration through the renal cortex
- D. Active transport of solutes

**Answer: B**

diffusion across membrane

Ch 1 | #89 | MCQ

**Which of the following conditions is associated with an increased risk of UTI due to urinary stasis?**

- A. Regular aerobic exercise
- B. Immobility and decreased fluid intake
- C. High-protein diet
- D. Use of oral contraceptives

**Answer: B**

both promote urinary stasis

Ch 1 | #90 | MCQ

**Dietary intervention is part of which aspect of CRF management?**

- A. Replacement therapy
- B. Surgical management
- C. Conservative (general) management
- D. Pharmacological management only

**Answer: C**

part of conservative care

## MATCHING

Ch 1 | #1 | MATCHING

**UTI a. Swelling caused by fluid retention b**

Ch 1 | #2 | MATCHING

**Cystitis b. Infection in any part of urinary tract c**

Ch 1 | #3 | MATCHING

**Hematuria c. Inflammation of the bladder d**

Ch 1 | #4 | MATCHING

**Nocturia d. Presence of blood in urine e**

Ch 1 | #5 | MATCHING

**Edema e. Frequent urination at night a**

Ch 1 | #6 | MATCHING

**Renal failure f. Loss of kidney function f**

Ch 1 | #7 | MATCHING

**Hemodialysis g. Artificial cleansing of blood g**

Ch 1 | #8 | MATCHING

**Anuria h. Absence of urine output h**

Ch 1 | #9 | MATCHING

**Oliguria i. Decreased urine output i**

Ch 1 | #10 | MATCHING

**Polyuria j. Excessive urine output j**

Ch 1 | #11 | MATCHING

**Pruritus k. Itching sensation k**

Ch 1 | #12 | MATCHING

**Dysuria l. Painful urination l**

Ch 1 | #13 | MATCHING

**Dyspnea m. Difficulty breathing m**

Ch 1 | #14 | MATCHING

**Fatigue n. Extreme tiredness n**

Ch 1 | #15 | MATCHING

**Nausea o. Feeling of wanting to vomit o**

## ESSAY

Ch 1 | #91 | ESSAY

**Which symptom in CRF results from the kidney's inability to excrete hydrogen**

**Answer: ions? A. Hypertension B. Edema C. Kussmaul breathing due to metabolic acidosis D. Pruritus** Kidney cannot excrete hydrogen ions. Metabolic acidosis develops. Result: Kussmaul breathing.

Ch 1 | #92 | ESSAY

**Define urinary tract infection (UTI) and describe its causes, risk factors, and**

**Answer:** symptoms in detail. UTI = infection anywhere in urinary tract. Usually bacterial ascent, often E. coli. Risk factors: female anatomy, pregnancy, catheterization, stones, immobility, low fluids. Symptoms: dysuria, urgency, frequency, hematuria, cloudy urine, pelvic pain, fever.

Ch 1 | #93 | ESSAY

**Outline the nursing education that should be provided to a patient to prevent**

**Answer:** recurrent urinary tract infections. Drink plenty of fluids and void regularly. Wipe front to back; keep perineum clean and dry. Void after intercourse. Avoid caffeine/alcohol; wear cotton underwear.

Ch 1 | #94 | ESSAY

**Classify the causes of chronic renal failure into pre-renal, renal, and post-**

**Answer:** renal, and give two examples of each. Pre-renal: poor cardiac function, chronic liver failure. Renal: diabetic nephropathy, HTN nephrosclerosis. Post-renal: neurogenic bladder, bilateral obstruction.

Ch 1 | #95 | ESSAY

**Describe the signs and symptoms of chronic renal failure, organising them by**

**Answer:** body system. GI: anorexia, nausea, vomiting, diarrhea, hiccups. Respiratory: Kussmaul breathing. Skin: pruritus, discoloration, nail changes. CV/fluid: edema, hypertension, CHF, ascites. Neuro: fatigue, confusion, cramps, seizures, coma.

Ch 1 | #96 | ESSAY

**Describe the conservative management of chronic renal failure and explain the**

**Answer:** role of each component. Control underlying causes and preserve remaining renal function. Diet/fluid management and lab monitoring. Treat anemia, electrolytes, acidosis, bone disease. Prepare for replacement therapy when needed.

Ch 1 | #97 | ESSAY

**Define hemodialysis and describe how it is performed, including vascular**

**Answer:** access methods. Hemodialysis = artificial cleansing of blood. Blood passes through dialyzer membrane. Waste and extra fluid removed. Access via AV fistula/graft/catheter.

Ch 1 | #98 | ESSAY

**Compare and contrast acute renal failure and chronic renal failure.**

**Answer:** Acute renal failure: sudden, often reversible. Chronic renal failure: gradual, progressive, usually irreversible. Both may need dialysis.

Ch 1 | #99 | ESSAY

**What are the dietary restrictions recommended for patients with chronic renal**

**Answer:** failure? Explain the rationale for each. Restrict sodium and fluid if overloaded. Restrict potassium and phosphorus. Moderate protein as ordered. Support calcium/vitamin D for bone health.

Ch 1 | #100 | ESSAY

**Write a comprehensive nursing care plan for a patient with chronic renal**

**Answer:** failure including nursing diagnoses, goals, and interventions. Diagnoses: excess fluid volume, poor nutrition, activity intolerance, electrolyte imbalance risk. Monitor I&O, weight, edema, BP, respiratory status, labs. Provide diet, meds, skin care, education, and dialysis teaching. Source note: MCQ 31, 33, and 34 in the original file appear malformed; answers use the best-fit intended meaning.

## Chapter 2 - Respiratory Tract Disorders

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### TRUE / FALSE

Ch 2 | #1 | TRUE / FALSE

**Pneumonia is inflammation of the airway and alveoli caused by infectious agents.**

**Answer: T**

Ch 2 | #2 | TRUE / FALSE

**Inhalation pneumonia can be caused by gastric content.**

**Answer: T**

Ch 2 | #3 | TRUE / FALSE

**Smoking is not considered a predisposing factor for pneumonia.**

**Answer: F**

Ch 2 | #4 | TRUE / FALSE

**A depressed cough reflex increases the risk of pneumonia.**

**Answer: T**

Ch 2 | #5 | TRUE / FALSE

**Sudden onset and shaking chills are signs of pneumonia.**

**Answer: T**

Ch 2 | #6 | TRUE / FALSE

**Chest pain in pneumonia is usually relieved by coughing.**

**Answer: F**

Ch 2 | #7 | TRUE / FALSE

**Tachypnea in pneumonia may be accompanied by nasal flaring.**

**Answer: T**

Ch 2 | #8 | TRUE / FALSE

**Sputum culture is used as a diagnostic tool for pneumonia.**

**Answer: T**



Ch 2 | #9 | TRUE / FALSE

**Frequent turning of bedridden patients helps prevent pneumonia.**

**Answer: T**

Ch 2 | #10 | TRUE / FALSE

**Isolation of the patient is never necessary during the acute stage of pneumonia.**

**Answer: F**

Ch 2 | #11 | TRUE / FALSE

**An upright position helps obtain greater lung expansion in pneumonia patients.**

**Answer: T**

Ch 2 | #12 | TRUE / FALSE

**Warm fluid intake should be discouraged for pneumonia patients.**

**Answer: F**

Ch 2 | #13 | TRUE / FALSE

**Postural drainage is a nursing management technique for pneumonia.**

**Answer: T**

Ch 2 | #14 | TRUE / FALSE

**Hypotension and shock are potential complications of pneumonia.**

**Answer: T**

Ch 2 | #15 | TRUE / FALSE

**Atelectasis in pneumonia is caused by accumulated secretions.**

**Answer: T**

Ch 2 | #16 | TRUE / FALSE

**Pulmonary TB is a communicable bacterial infection.**

**Answer: T**

Ch 2 | #17 | TRUE / FALSE

**TB is caused by the Mycobacterium tuberculosis bacteria.**

**Answer: T**

Ch 2 | #18 | TRUE / FALSE

**TB can only affect the lungs and no other organs.**

**Answer: F**

Ch 2 | #19 | TRUE / FALSE

**Primary stage TB infection is usually asymptomatic.**

**Answer: T**

Ch 2 | #20 | TRUE / FALSE

**Drug-resistant bacteria can emerge from incomplete TB treatment.**

**Answer: T**

Ch 2 | #21 | TRUE / FALSE

**Close contact with an active TB patient is a risk factor.**

**Answer: T**

Ch 2 | #22 | TRUE / FALSE

**Children under age 15 are not at risk for TB.**

**Answer: F**

Ch 2 | #23 | TRUE / FALSE

**Night sweats and weight loss are systemic symptoms of TB.**

**Answer: T**

Ch 2 | #24 | TRUE / FALSE

**Hemoptysis means coughing up blood.**

**Answer: T**

Ch 2 | #25 | TRUE / FALSE

**TB is spread through aerosol droplets from coughing or sneezing.**

**Answer: T**

Ch 2 | #26 | TRUE / FALSE

**Latent TB patients can transmit the disease to others.**

**Answer: F**

Ch 2 | #27 | TRUE / FALSE

**A tuberculin skin test is used in TB diagnosis.**

**Answer: T**

Ch 2 | #28 | TRUE / FALSE

**Rifampicin and Isoniazid are common antibiotics for TB.**

**Answer: T**

Ch 2 | #29 | TRUE / FALSE

**There is a reliable TB vaccine available for all adults.**

**Answer: F**

Ch 2 | #30 | TRUE / FALSE

**Hospitalised TB patients should be kept in negative pressure rooms.**

**Answer: T**

Ch 2 | #31 | TRUE / FALSE

**COPD airflow limitation is fully reversible.**

**Answer: F**

Ch 2 | #32 | TRUE / FALSE

**Emphysema and chronic bronchitis are types of COPD.**

**Answer: T**

Ch 2 | #33 | TRUE / FALSE

**Genetic deficiency of alpha antitrypsin is a risk factor for COPD.**

**Answer: T**

Ch 2 | #34 | TRUE / FALSE

**Tobacco smoke is a primary risk factor for COPD.**

**Answer: T**

Ch 2 | #35 | TRUE / FALSE

**Tachypnea is a clinical manifestation of COPD.**

**Answer: T**

Ch 2 | #36 | TRUE / FALSE

**In COPD, breathing out takes a shorter time than breathing in.**

**Answer: F**

Ch 2 | #37 | TRUE / FALSE

**Pursed-lip breathing is a sign/technique used by COPD patients.**

**Answer: T**

Ch 2 | #38 | TRUE / FALSE

**Finger clubbing can be a sign of emphysema.**

**Answer: T**

Ch 2 | #39 | TRUE / FALSE

**Quitting smoking is a medical management strategy for COPD.**

**Answer: T**

Ch 2 | #40 | TRUE / FALSE

**Bronchodilators are used to dilate the airways.**

**Answer: T**

Ch 2 | #41 | TRUE / FALSE

**Patients with COPD should avoid dust-free environments.**

**Answer: F**

Ch 2 | #42 | TRUE / FALSE

**High fluid intake (2–3 litres) is recommended for COPD patients.**

**Answer: T**

Ch 2 | #43 | TRUE / FALSE

**Dairy products should be avoided if they increase sputum production.**

**Answer: T**

Ch 2 | #44 | TRUE / FALSE

**Tachycardia is a potential adverse effect of bronchodilators.**

**Answer: T**

Ch 2 | #45 | TRUE / FALSE

**Right-sided heart failure is a complication of COPD.****Answer: T**

## MCQ

Ch 2 | #46 | MCQ

**Which type of pneumonia is caused by inhaling gastric content?**

- A. Viral
- B. Fungal
- C. Inhalation
- D. Bacterial

**Answer: C**

aspiration from gastric contents

Ch 2 | #47 | MCQ

**Which of the following is a predisposing factor for pneumonia?**

- A. Exercise
- B. Immunosuppression
- C. High protein diet
- D. Youth

**Answer: B**

weak immunity predisposes infection

Ch 2 | #48 | MCQ

**A common sign of pneumonia is sputum that is:**

- A. Clear
- B. Frothy
- C. Purulent
- D. Absent

**Answer: C**

purulent sputum is common

Ch 2 | #49 | MCQ

**Diagnostic tests for pneumonia include:**

- A. MRI
- B. Chest X-ray
- C. EEG
- D. Bone scan

**Answer: B**

standard imaging for pneumonia

Ch 2 | #50 | MCQ

**To prevent aspiration in unconscious patients, the nurse should:**

- A. Give oral fluids
- B. Suction secretions
- C. Keep them flat
- D. Avoid turning

**Answer: B**

clears secretions, lowers aspiration risk

Ch 2 | #51 | MCQ

**What is the priority nursing action for a pneumonia patient with inadequate gas exchange?**

- A. Give a bath
- B. Oxygen therapy
- C. Exercise
- D. Solid food

**Answer: B**

corrects hypoxemia first

Ch 2 | #52 | MCQ

**The best position for a patient with pneumonia to expand lungs is:**

- A. Upright
- B. Supine
- C. Prone
- D. Side-lying

**Answer: A**

maximizes lung expansion

Ch 2 | #53 | MCQ

**Which of these helps loosen secretions in pneumonia?**

- A. Dehydration
- B. Bed rest
- C. Warm fluid intake
- D. Cold air

**Answer: C**

helps thin and loosen mucus

Ch 2 | #54 | MCQ

**A complication of pneumonia where fluid accumulates around the lungs is:**

- A. Shock
- B. Pleural effusion
- C. Hypertension
- D. Obesity

**Answer: B**

fluid collects in pleura

Ch 2 | #55 | MCQ

**Nursing management for pneumonia includes monitoring:**

- A. Vital signs
- B. Eye sight
- C. Hearing
- D. Hair growth

**Answer: A**

tracks fever, pulse, respiration

Ch 2 | #56 | MCQ

**Frequent suctioning can be a:**

- A. Predisposing factor
- B. Cure
- C. Diagnostic test
- D. Vaccine

**Answer: A**

invasive suction raises infection risk

Ch 2 | #57 | MCQ

**Antimicrobial therapy choice depends on:**

- A. Patient's age
- B. Sputum culture
- C. Weight
- D. Gender

**Answer: B**

guides targeted antimicrobial choice

Ch 2 | #58 | MCQ

**Preventive measures for pneumonia include:**

- A. Smoking
- B. Immunization
- C. Immobility
- D. Shallow breathing

**Answer: B**

vaccines lower pneumonia risk

Ch 2 | #59 | MCQ

**ABGs are followed to determine:**

- A. Blood type
- B. Oxygen need
- C. Glucose level
- D. Bone density

**Answer: B**

assesses oxygenation requirements

Ch 2 | #60 | MCQ

**Which drug relieves rhinorrhea in pneumonia?**

- A. Antibiotics
- B. Antihistamines
- C. Aspirin
- D. Vitamin C

**Answer: B**

reduces rhinorrhoea and sneezing

Ch 2 | #61 | MCQ

**TB is primarily an infection of the:**

- A. Liver
- B. Lungs
- C. Heart
- D. Skin

**Answer: B**

primary pulmonary site



Ch 2 | #62 | MCQ

**How is TB transmitted?**

- A. Skin contact
- B. Aerosol droplets
- C. Contaminated water
- D. Blood transfusion

**Answer: B**

spread by inhaled droplet nuclei

Ch 2 | #63 | MCQ

**A cough suspicious of TB usually lasts more than:**

- A. 1 week
- B. 2 weeks
- C. 3 weeks
- D. 1 day

**Answer: C**

persistent cough over three weeks

Ch 2 | #64 | MCQ

**Systemic symptoms of TB include:**

- A. Weight gain
- B. High energy
- C. Night sweats
- D. Increased appetite

**Answer: C**

classic constitutional TB symptom

Ch 2 | #65 | MCQ

**Extra-pulmonary TB can affect the:**

- A. Central nervous system
- B. Hair
- C. Nails
- D. Teeth

**Answer: A**

TB may involve CNS

Ch 2 | #66 | MCQ

**Which antibiotic is commonly used to treat TB?**

- A. Amoxicillin
- B. Isoniazid
- C. Insulin
- D. Warfarin

**Answer: B**

first-line anti-TB drug

Ch 2 | #67 | MCQ

**To promote airway clearance in TB, the nurse should encourage:**

- A. Fluid intake
- B. Fluid restriction
- C. Bed rest only
- D. Fasting

**Answer: A**

thins secretions for expectoration

Ch 2 | #68 | MCQ

**Pyridoxine (Vitamin B6) is given to TB patients to prevent:**

- A. Peripheral neuropathy
- B. Blindness
- C. Scurvy
- D. Rickets

**Answer: A**

prevents isoniazid neuropathy

Ch 2 | #69 | MCQ

**TB patients are taught to cover their mouth when:**

- A. Eating
- B. Sleeping
- C. Coughing
- D. Walking

**Answer: C**

limits airborne transmission

Ch 2 | #70 | MCQ

**Which room type is required for a hospitalized active TB patient?**

- A. Standard room
- B. Negative pressure room
- C. Suite
- D. Intensive care

**Answer: B**

airborne isolation requires negative pressure

Ch 2 | #71 | MCQ

**TB can spread through the:**

- A. Hair
- B. Lymph fluid
- C. Sweat
- D. Tears

**Answer: B**

can disseminate via lymphatics

Ch 2 | #72 | MCQ

**A complete TB evaluation includes:**

- A. Eye exam
- B. Medical history
- C. Hearing test
- D. Stress test

**Answer: B**

history is part of evaluation

Ch 2 | #73 | MCQ

**TB prevention in children involves:**

- A. Exercise
- B. Vaccination
- C. Dieting
- D. Antibiotics

**Answer: B**

BCG helps protect children

Ch 2 | #74 | MCQ

**Monitoring weight in TB patients is part of:**

- A. Airway clearance
- B. Promoting nutrition
- C. Hygiene
- D. Diagnosis

**Answer: B**

reflects nutritional status

Ch 2 | #75 | MCQ

**A complication of TB is involvement of the:**

- A. Kidneys
- B. Eyes
- C. Ears
- D. Skin

**Answer: A**

extrapulmonary TB may affect kidneys

Ch 2 | #76 | MCQ

**COPD is characterized by:**

- A. Reversible airflow
- B. Airflow limitation
- C. High blood pressure
- D. Strong lungs

**Answer: B**

hallmark is chronic airflow limitation

Ch 2 | #77 | MCQ

**A major occupational risk factor for COPD is:**

- A. Office work
- B. Chemical fumes
- C. Teaching
- D. Coding

**Answer: B**

inhaled fumes damage airways

Ch 2 | #78 | MCQ

**Which enzyme inhibitor deficiency causes lung injury in COPD?**

- A. Insulin
- B. Alpha antitrypsin
- C. Pepsin
- D. Lipase

**Answer: B**

deficiency predisposes emphysema

Ch 2 | #79 | MCQ

**A physical sign of COPD is:**

- A. Small chest
- B. Enlargement of chest
- C. Slow heart rate
- D. Weight gain

**Answer: B**

barrel chest from hyperinflation

Ch 2 | #80 | MCQ

**Pursed-lip breathing helps to:**

- A. Control rate of respiration
- B. Stop coughing
- C. Increase fever
- D. Reduce thirst

**Answer: A**

prolongs exhalation, eases breathing

Ch 2 | #81 | MCQ

**Which medication decreases airway inflammation in COPD?**

- A. Bronchodilators
- B. Corticosteroids
- C. Antibiotics
- D. Antihistamines

**Answer: B**

suppresses airway inflammation

Ch 2 | #82 | MCQ

**Pulmonary rehabilitation involves:**

- A. Education on lung function
- B. Surgery
- C. Weight lifting
- D. Fasting

**Answer: A**

rehab includes education and training

Ch 2 | #83 | MCQ

**To clear secretions in COPD, the nurse uses:**

- A. Bed rest
- B. Postural drainage
- C. Cold compresses
- D. Massage

**Answer: B**

mobilizes retained secretions

Ch 2 | #84 | MCQ

**Nebulized saline inhalations help to:**

- A. Stop pain
- B. Clear secretions
- C. Reduce fever
- D. Help sleep

**Answer: B**

moistens and loosens mucus

Ch 2 | #85 | MCQ

**COPD patients should monitor for tachycardia when taking:**

- A. Oxygen
- B. Bronchodilators
- C. Water
- D. Vitamins

**Answer: B**

bronchodilators may cause tachycardia

Ch 2 | #86 | MCQ

**A technique to manage fatigue in COPD is:**

- A. Energy conservation
- B. Running
- C. Skipping meals
- D. Smoking

**Answer: A**

reduces oxygen demand

Ch 2 | #87 | MCQ

**A sign of emphysema is:**

- A. Red skin
- B. Central cyanosis
- C. High energy
- D. Thick hair

**Answer: B**

sign of advanced hypoxemia

Ch 2 | #88 | MCQ

**COPD can lead to which skeletal issue?**

- A. Strong bones
- B. Muscle dysfunction
- C. Increased height
- D. Fever

**Answer: B**

COPD causes muscle dysfunction

Ch 2 | #89 | MCQ

**In COPD, wheezing is heard through a:**

- A. Thermometer
- B. Stethoscope
- C. Otoscope
- D. Sphygmomanometer

**Answer: B**

wheezes heard on auscultation

Ch 2 | #90 | MCQ

**Depression is a possible \_\_\_\_\_ of COPD.**

- A. Cause
- B. Risk factor
- C. Complication
- D. Cure

**Answer: C**

depression is a complication

## MATCHING

Ch 2 | #1 | MATCHING

**Pneumonia (bacterial) A. Mycobacterium tuberculosis C**

Ch 2 | #2 | MATCHING

**Pulmonary Tuberculosis B. Aspiration of gastric content A**

Ch 2 | #3 | MATCHING

**Fungal Pneumonia C. Streptococcus bacteria E**

Ch 2 | #4 | MATCHING

**Inhalation Pneumonia D. Long-term tobacco smoke exposure B**

Ch 2 | #5 | MATCHING

**COPD E. Candida or Aspergillus species D**

Ch 2 | #6 | MATCHING

**Rifampicin and Isoniazid A. Dilate airways and improve airflow in COPD D**

Ch 2 | #7 | MATCHING

**Bronchodilators B. Relieve rhinorrhoea and sneezing in pneumonia A**

Ch 2 | #8 | MATCHING

**Antihistaminics C. Protect high-risk persons from pneumonia B**

Ch 2 | #9 | MATCHING

**BCG Vaccine D. Kill Mycobacterium tuberculosis E**



Ch 2 | #10 | MATCHING

**Pneumococcal Vaccine E. Protect children against tuberculosis C**

## ESSAY

Ch 2 | #91 | ESSAY

**Define Pneumonia and list its four classifications.**

**Answer: Pneumonia = inflammation/infection of lung parenchyma and alveoli.**  
**Classifications: bacterial, viral, fungal, aspiration/inhalation.**

Ch 2 | #92 | ESSAY

**List five predisposing factors for pneumonia.**

**Answer: Smoking. Immunosuppression/chronic illness. Depressed cough reflex. Bed rest/immobility. Aspiration risk.**

Ch 2 | #93 | ESSAY

**What are the preventive measures for pneumonia?**

**Answer: Immunization and hand hygiene. Frequent turning, coughing, deep breathing, early ambulation. Oral care and aspiration precautions. Smoking cessation and hydration.**

Ch 2 | #94 | ESSAY

**Explain how Pulmonary Tuberculosis is transmitted.**

**Answer: TB spreads through airborne droplet nuclei. Released by coughing, sneezing, talking, singing. Others inhale droplets into lungs. Risk rises with crowding and poor ventilation.**

Ch 2 | #95 | ESSAY

**Identify five risk factors for developing Tuberculosis.**

**Answer: Close contact with active TB case. HIV/immunosuppression. Malnutrition. Overcrowding/poor ventilation. Diabetes/chronic illness/extremes of age.**

Ch 2 | #96 | ESSAY

**What are the systemic symptoms and common diagnostic tests for TB?**

**Answer: Symptoms: fever, night sweats, weight loss, fatigue, anorexia. Tests: TST/IGRA, sputum AFB/culture, chest X-ray, molecular testing.**

Ch 2 | #97 | ESSAY

**Define COPD and list its primary risk factors.**

**Answer:** COPD = chronic progressive airflow limitation not fully reversible. Includes chronic bronchitis and emphysema. Main risks: smoking, biomass smoke, air pollution, dust/fumes, alpha-1 deficiency.

Ch 2 | #98 | ESSAY

**Describe the nursing interventions to improve airway clearance in a COPD**

**Answer:** patient. Assess breath sounds, sputum, RR, SpO2. High Fowler's; pursed-lip and huff coughing. 2–3 L/day fluids if allowed. Bronchodilators/nebulization; chest physiotherapy/postural drainage.

Ch 2 | #99 | ESSAY

**List five potential complications of COPD.**

**Answer:** Respiratory failure. Recurrent infection/pneumonia. Pulmonary hypertension. Cor pulmonale/right heart failure. Depression, malnutrition, muscle dysfunction.

Ch 2 | #100 | ESSAY

**Write a nursing care plan for a COPD patient including diagnoses, goals, and**

**Answer:** interventions. Diagnoses: ineffective airway clearance, impaired gas exchange, activity intolerance. Goals: maintain airway, improve oxygenation, reduce dyspnea. Interventions: upright position, monitor ABGs/SpO2, airway clearance, oxygen and meds, smoking cessation teaching.

## Chapter 3 - Digestive System Disorders

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### TRUE / FALSE

Ch 3 | #1 | TRUE / FALSE

**Peptic ulcers can form in the esophagus.**

**Answer: T**

Ch 3 | #2 | TRUE / FALSE

**Gastric ulcers are most commonly found in the greater curvature of the stomach.**

**Answer: F**

Ch 3 | #3 | TRUE / FALSE

**H. pylori infection plays a role in the development of gastric ulcers.**

**Answer: T**

Ch 3 | #4 | TRUE / FALSE

**Duodenal ulcers are often associated with protein-rich meals.**

**Answer: T**

Ch 3 | #5 | TRUE / FALSE

**Stress ulcers usually occur after a chronic illness rather than an acute crisis.**

**Answer: F**

Ch 3 | #6 | TRUE / FALSE

**In gastric ulcers, acid secretion is typically hyper-secretion.**

**Answer: F**

Ch 3 | #7 | TRUE / FALSE

**Pain from a duodenal ulcer usually occurs 2–4 hours after meals.**

**Answer: T**

Ch 3 | #8 | TRUE / FALSE

**Food typically aggravates the pain in a gastric ulcer.**

**Answer: T**

Ch 3 | #9 | TRUE / FALSE

**Vomiting is a common symptom of duodenal ulcers.**

**Answer: F**

Ch 3 | #10 | TRUE / FALSE

**Duodenal ulcers are more likely to manifest as haematemesis than gastric ulcers.**

**Answer: F**

Ch 3 | #11 | TRUE / FALSE

**Melaena is more common with duodenal ulcers.**

**Answer: T**

Ch 3 | #12 | TRUE / FALSE

**Fibre optic endoscopes allow direct visualisation of the stomach and duodenum.**

**Answer: T**

Ch 3 | #13 | TRUE / FALSE

**Patients should limit fluids for 48 hours after a barium X-ray.**

**Answer: F**

Ch 3 | #14 | TRUE / FALSE

**Barium may cause the stool to appear white for several days.**

**Answer: T**

Ch 3 | #15 | TRUE / FALSE

**Haemorrhage is a potential complication of peptic ulcers.**

**Answer: T**

Ch 3 | #16 | TRUE / FALSE

**Orthostatic hypotension is a manifestation of ulcer haemorrhage.**

**Answer: T**

Ch 3 | #17 | TRUE / FALSE

**"Coffee ground" vomitus is more likely associated with a gastric ulcer.**

**Answer: T**

Ch 3 | #18 | TRUE / FALSE

**A saline lavage via NGT is used during the management of haemorrhage.**

**Answer: T**

Ch 3 | #19 | TRUE / FALSE

**Perforation is when GI contents empty into the peritoneal cavity.**

**Answer: T**

Ch 3 | #20 | TRUE / FALSE

**The pain from perforation is described as dull and localised.**

**Answer: F**

Ch 3 | #21 | TRUE / FALSE

**A rigid, tender abdomen is a sign of perforation.**

**Answer: T**

Ch 3 | #22 | TRUE / FALSE

**Perforation is considered a surgical emergency.**

**Answer: T**

Ch 3 | #23 | TRUE / FALSE

**Pyloric obstruction is caused by excessive acid only.**

**Answer: F**

Ch 3 | #24 | TRUE / FALSE

**Persistent vomiting is a symptom of pyloric obstruction.**

**Answer: T**

Ch 3 | #25 | TRUE / FALSE

**Pyloric obstruction can lead to metabolic acidosis.**

**Answer: F**

Ch 3 | #26 | TRUE / FALSE

**Conservative therapy for ulcers includes the elimination of smoking.**

**Answer: T**

Ch 3 | #27 | TRUE / FALSE

**Antibiotics are used to eradicate H. pylori infections.**

**Answer: T**

Ch 3 | #28 | TRUE / FALSE

**Recurrence of ulcers is much higher (75–90%) if H. pylori is not treated.**

**Answer: T**

Ch 3 | #29 | TRUE / FALSE

**Antacids are the initial drugs of choice for medical management.**

**Answer: T**

Ch 3 | #30 | TRUE / FALSE

**Histamine H2 receptor antagonists increase HCl secretion.**

**Answer: F**

Ch 3 | #31 | TRUE / FALSE

**Anticholinergic drugs speed up gastric motility.**

**Answer: F**

Ch 3 | #32 | TRUE / FALSE

**Gastroduodenostomy involves connecting the stomach to the jejunum.**

**Answer: F**

Ch 3 | #33 | TRUE / FALSE

**Gastrojejunostomy is the removal of the lower stomach and connecting it to the jejunum.**

**Answer: T**

Ch 3 | #34 | TRUE / FALSE

**Total gastrectomy involves the removal of the entire stomach.**

**Answer: T**

Ch 3 | #35 | TRUE / FALSE

**In a total gastrectomy, the esophagus is connected directly to the jejunum.**

**Answer: T**

Ch 3 | #36 | TRUE / FALSE

**Subtotal gastrectomy involves removing the distal third of the stomach.**

**Answer: T**

Ch 3 | #37 | TRUE / FALSE

**Hypokalaemia is a risk in patients with pyloric obstruction.**

**Answer: T**

Ch 3 | #38 | TRUE / FALSE

**The "back diffusion" of HCl damages the gastric epithelium.**

**Answer: T**

Ch 3 | #39 | TRUE / FALSE

**Bile reflux from the duodenum cannot damage the gastric mucosal barrier.**

**Answer: F**

Ch 3 | #40 | TRUE / FALSE

**Duodenal ulcers lead to a large acid load in the duodenum.**

**Answer: T**

Ch 3 | #41 | TRUE / FALSE

**Ischaemia is a contributing factor to the evolution of stress ulcers.**

**Answer: T**

Ch 3 | #42 | TRUE / FALSE

**Upper GI series uses barium to detect structural disorders.**

**Answer: T**

Ch 3 | #43 | TRUE / FALSE

**NGT suction helps prevent distension during haemorrhage management.**

**Answer: T**

Ch 3 | #44 | TRUE / FALSE

**Perforation management includes keeping the patient NPO (nothing by mouth).**

**Answer: T**

Ch 3 | #45 | TRUE / FALSE

**Stomach emptying is enhanced in pyloric obstruction.****Answer: F**

## MCQ

Ch 3 | #46 | MCQ

**Which of the following is NOT a location for a peptic ulcer?**

- A. Stomach
- B. Duodenum
- C. Esophagus
- D. Descending colon

**Answer: D**

not a peptic-ulcer site

Ch 3 | #47 | MCQ

**Gastric ulcers are most common near the:**

- A. Fundus
- B. Pylorus
- C. Cardia
- D. Esophagus

**Answer: B**

common distal gastric location

Ch 3 | #48 | MCQ

**Which bacteria is heavily linked to peptic ulcers?**

- A. E. coli
- B. Streptococcus
- C. H. pylori
- D. Staphylococcus

**Answer: C**

classic ulcer-associated bacterium

Ch 3 | #49 | MCQ

**What is the primary cause of duodenal ulcers?**

- A. Excessive acid
- B. Lack of acid
- C. Rapid stomach emptying only
- D. Excessive mucus

**Answer: A**

excess acid load



Ch 3 | #50 | MCQ

**Stress ulcers are most common in which parts of the GI tract?**

- A. Esophagus and pylorus
- B. Proximal stomach and duodenum
- C. Distal stomach and jejunum
- D. Ileum and colon

**Answer: B**

usual stress-ulcer locations

Ch 3 | #51 | MCQ

**Pain that occurs 1-2 hours after a meal is characteristic of:**

- A. Gastric ulcer
- B. Duodenal ulcer
- C. Stress ulcer
- D. Pyloric obstruction

**Answer: A**

pain 1–2 hours postmeal

Ch 3 | #52 | MCQ

**Which ulcer type is often relieved by food?**

- A. Gastric
- B. Duodenal
- C. Esophageal
- D. Stress

**Answer: B**

food relieves duodenal pain

Ch 3 | #53 | MCQ

**Hematemesis is more likely to occur with which type of ulcer?**

- A. Gastric
- B. Duodenal
- C. Stress
- D. All of the above

**Answer: A**

haematemesis more gastric

Ch 3 | #54 | MCQ

**Melena refers to:**

- A. Bloody vomit
- B. Black, tarry stool
- C. Clear liquid discharge
- D. Excessive gas

**Answer: B**

black tarry stool

Ch 3 | #55 | MCQ

**The procedure used to visualize the esophagus, stomach, and duodenum directly is:**

- A. Upper GI series
- B. Esophagogastroduodenoscopy
- C. Abdominal X-ray
- D. Colonoscopy

**Answer: B**

direct upper-GI visualization

Ch 3 | #56 | MCQ

**Following an Upper GI series, why is a mild laxative sometimes recommended?**

- A. To stop diarrhea
- B. If constipation occurs due to barium
- C. To increase acid production
- D. To help with pain

**Answer: B**

barium may constipate

Ch 3 | #57 | MCQ

**A "coffee ground" appearance of vomitus indicates:**

- A. Fresh bleeding
- B. Older blood affected by stomach acid
- C. High bile content
- D. Normal digestion

**Answer: B**

digested blood in vomitus

Ch 3 | #58 | MCQ

**Signs of orthostatic hypotension during hemorrhage include:**

- A. High blood pressure when sitting
- B. Drop in BP and dizziness upon standing
- C. Flushed, warm skin
- D. Bradycardia

**Answer: B**

postural BP drop, dizziness

Ch 3 | #59 | MCQ

**Management of hemorrhage includes all the following EXCEPT:**

- A. NGT insertion
- B. Saline lavage
- C. High-fiber diet
- D. Fluid replacement

**Answer: C**

not haemorrhage management

Ch 3 | #60 | MCQ

**Perforation leads to the emptying of GI contents into the:**

- A. Esophagus
- B. Peritoneal cavity
- C. Pleural cavity
- D. Small intestine only

**Answer: B**

contents spill intraperitoneally

Ch 3 | #61 | MCQ

**A patient with a rigid, board-like abdomen likely has:**

- A. Pyloric obstruction
- B. Simple gastric ulcer
- C. Perforation
- D. Stress ulcer

**Answer: C**

board-like abdomen suggests perforation

Ch 3 | #62 | MCQ

**Which is a medical emergency requiring urgent surgery?**

- A. Duodenal ulcer pain
- B. Gastric ulcer pain
- C. Perforation
- D. Pyloric inflammation

**Answer: C**

needs urgent surgery

Ch 3 | #63 | MCQ

**Pyloric obstruction often causes which electrolyte imbalance?**

- A. Hyperkalemia
- B. Hypokalemia
- C. Hyponatremia
- D. Hypercalcemia

**Answer: B**

flawed item; associated imbalance

Ch 3 | #64 | MCQ

**Metabolic alkalosis in pyloric obstruction is caused by:**

- A. Excessive diarrhea
- B. Persistent vomiting of stomach acid
- C. Rapid digestion
- D. Antibiotic therapy

**Answer: B**

flawed item; closest cause

Ch 3 | #65 | MCQ

**Conservative therapy for ulcers includes:**

- A. Increased smoking
- B. Physical and emotional rest
- C. Spicy food diet
- D. Avoiding follow-up care

**Answer: B**

rest is conservative care

Ch 3 | #66 | MCQ

**The primary goal of using antibiotics in ulcer treatment is:**

- A. To reduce acid
- B. To eradicate H. pylori
- C. To stop bleeding
- D. To speed up gastric emptying

**Answer: B**

eradicates H. pylori

Ch 3 | #67 | MCQ

**Initial drugs of choice for medical management are:**

- A. Antibiotics
- B. Antacids
- C. Anticholinergics
- D. Histamine blockers

**Answer: B**

traditional first-line neutralizer

Ch 3 | #68 | MCQ

**Which medication type stops cholinergic stimulation of HCl?**

- A. H2 blockers
- B. Anticholinergics
- C. Antacids
- D. Antibiotics

**Answer: B**

blocks cholinergic acid stimulation

Ch 3 | #69 | MCQ

**Gastroduodenostomy connects the remaining stomach to the:**

- A. Jejunum
- B. Ileum
- C. Duodenum
- D. Esophagus

**Answer: C**

remnant joins duodenum

Ch 3 | #70 | MCQ

**Subtotal gastrectomy removes how much of the stomach?**

- A. Proximal half
- B. Distal third
- C. The entire stomach
- D. Only the pylorus

**Answer: B**

distal third removed

Ch 3 | #71 | MCQ

**Which surgical procedure connects the esophagus to the jejunum?**

- A. Subtotal gastrectomy
- B. Gastroduodenostomy
- C. Total gastrectomy
- D. Gastrojejunostomy

**Answer: C**

esophagus joined to jejunum

Ch 3 | #72 | MCQ

**Back diffusion refers to HCl damaging the:**

- A. Gastric epithelium
- B. Duodenal wall only
- C. Esophageal sphincter
- D. Pyloric valve

**Answer: A**

flawed item; intended mucosal damage

Ch 3 | #73 | MCQ

**Protein-rich meals contribute to duodenal ulcers because they:**

- A. Stimulate acid production
- B. Block the pylorus
- C. Neutralize all acid
- D. Kill H. pylori

**Answer: A**

protein stimulates acid secretion

Ch 3 | #74 | MCQ

**Which ulcer type is most associated with rapid emptying of food?**

- A. Gastric
- B. Duodenal
- C. Stress
- D. Esophageal

**Answer: B**

linked with rapid emptying

Ch 3 | #75 | MCQ

**Ischemia is a major factor in:**

- A. Gastric ulcers
- B. Duodenal ulcers
- C. Stress ulcers
- D. Pyloric obstruction

**Answer: C**

ischemia drives stress ulcers

Ch 3 | #76 | MCQ

**In an Upper GI series, the patient ingests:**

- A. Saline
- B. Barium
- C. Dye via IV
- D. Antibiotics

**Answer: B**

barium contrast is swallowed

Ch 3 | #77 | MCQ

**Clinical manifestations of hemorrhage include:**

- A. Warm, dry skin
- B. Cool, clammy skin
- C. Hypertension
- D. Slow heart rate

**Answer: B**

hypovolemic cool clammy skin

Ch 3 | #78 | MCQ

**The purpose of NGT suction in hemorrhage is to:**

- A. Feed the patient
- B. Increase acid production
- C. Prevent distension and assess bleeding rate
- D. Deliver oxygen

**Answer: C**

decompresses stomach, tracks bleeding

Ch 3 | #79 | MCQ

**Perforation pain can spread to:**

- A. Only the chest
- B. Only the legs
- C. All of the abdomen
- D. The arms

**Answer: C**

pain becomes generalized

Ch 3 | #80 | MCQ

**Which is NOT a management step for perforation?**

- A. NPO
- B. Antibiotics
- C. High-fat diet
- D. I & O monitoring

**Answer: C**

fat is not treatment

Ch 3 | #81 | MCQ

**Pyloric obstruction is caused by:**

- A. Excessive exercise
- B. Inflammation or edema
- C. Low acid secretion
- D. Rapid stomach emptying

**Answer: B**

due to edema/inflammation



Ch 3 | #82 | MCQ

**Abdominal bloating is a symptom of:**

- A. Gastric ulcer
- B. Duodenal ulcer
- C. Pyloric obstruction
- D. Stress ulcer

**Answer: C**

gastric outlet obstruction symptom

Ch 3 | #83 | MCQ

**Histamine H2 receptor antagonists work by:**

- A. Neutralizing existing acid
- B. Blocking the activator of HCl secretion
- C. Eradicating H. pylori
- D. Coating the ulcer

**Answer: B**

blocks acid-secretory activator

Ch 3 | #84 | MCQ

**Gastrojejunostomy is also known as a procedure that reconnects the:**

- A. Esophagus to duodenum
- B. Stomach to duodenum
- C. Stomach to jejunum
- D. Duodenum to jejunum

**Answer: C**

stomach reconnected to jejunum

Ch 3 | #85 | MCQ

**Dietary modifications are part of:**

- A. Conservative therapy
- B. Surgical therapy
- C. Emergency management only
- D. Diagnosis

**Answer: A**

part of conservative care

Ch 3 | #86 | MCQ

**The recurrence rate of ulcers with H. pylori infection is:**

- A. 10-20%
- B. 40-50%
- C. 75-90%
- D. 100%

**Answer: A**

recurrence lower after treatment

Ch 3 | #87 | MCQ

**Total gastrectomy is the:**

- A. Partial removal of the stomach
- B. Full removal of the stomach
- C. Removal of the duodenum
- D. Removal of the pylorus

**Answer: B**

complete stomach removal

Ch 3 | #88 | MCQ

**Management of hemorrhage requires replacing:**

- A. Oxygen only
- B. Fiber
- C. Blood, fluid, and electrolytes
- D. Antibiotics only

**Answer: C**

replace blood, fluids, electrolytes

Ch 3 | #89 | MCQ

**Bright red hematemesis is more likely in:**

- A. Gastric ulcer
- B. Duodenal ulcer
- C. Pyloric obstruction
- D. Mild gastritis

**Answer: A**

upper gastric bleeding pattern

Ch 3 | #90 | MCQ

**Peptic ulcers are essentially:**

- A. Tumors
- B. Erosions in the mucosal wall
- C. Infections of the outer wall
- D. Muscle tears

**Answer: B**

mucosal erosive lesion

## MATCHING

Ch 3 | #1 | MATCHING

**Gastric Ulcer A. Manifests as rigid, tender abdomen and sudden sharp pain C**

Ch 3 | #2 | MATCHING

**Duodenal Ulcer B. Involves reconnecting stomach to jejunum after removing part D**

Ch 3 | #3 | MATCHING

**Perforation C. Characterised by pain 1–2 hours after meals; vomiting common A**

Ch 3 | #4 | MATCHING

**Pyloric Obstruction D. Pain often relieved by food; may be relieved by food E**

Ch 3 | #5 | MATCHING

**Gastrojejunostomy E. Leads to abdominal bloating, persistent vomiting, hypokalaemia B**

Ch 3 | #6 | MATCHING

**Stress Ulcer C. Occurs after acute medical crisis, trauma, or major surgery C**

Ch 3 | #7 | MATCHING

**Upper GI Series E. Diagnostic test involving barium ingestion; may cause white stools E**

Ch 3 | #8 | MATCHING

**Haematemesis D. Bloody or coffee-ground vomit — more common with gastric ulcers D**

Ch 3 | #9 | MATCHING

**H. pylori C. Bacterium — major cause of ulcer recurrence C**

Ch 3 | #10 | MATCHING

**H2 Receptor Antagonists D. Block final intracellular activator of HCl secretion D**

## ESSAY

Ch 3 | #91 | ESSAY

**Define peptic ulcer and list the three main types.**

**Answer: Peptic ulcer = mucosal erosion from acid-pepsin injury. Main types: gastric, duodenal, stress ulcers. May also occur in lower esophagus.**

Ch 3 | #92 | ESSAY

**Compare the timing and nature of pain between Gastric and Duodenal ulcers.**

**Answer: Gastric ulcer pain: 1–2 h after meals and worsens with food. Duodenal pain: 2–4 h after meals/night and relieved by food or antacids.**

Ch 3 | #93 | ESSAY

**Explain the role of H. pylori in the development of peptic ulcers.**

**Answer: H. pylori inflames gastric mucosa. Weakens protective barrier. Increases susceptibility to acid-pepsin injury. Raises recurrence if untreated.**

Ch 3 | #94 | ESSAY

**What is the "Upper GI series" and what are the nursing responsibilities after**

**Answer: the procedure? Upper GI series = barium X-ray of esophagus/stomach/duodenum. Encourage fluids after test. Laxative may be needed. Pale stools may occur briefly.**

Ch 3 | #95 | ESSAY

**Describe the clinical manifestations of ulcer perforation.**

**Answer: Sudden severe sharp epigastric pain. Rigid board-like abdomen and tenderness. Absent bowel sounds, tachycardia, shock/peritonitis signs.**

Ch 3 | #96 | ESSAY

**Outline the management steps for a patient experiencing a GI haemorrhage.**

**Answer: NPO and IV access. Replace blood, fluids, electrolytes. Insert NGT if ordered. Monitor vitals, urine, bleeding. Prepare for endoscopy/surgery if uncontrolled.**

Ch 3 | #97 | ESSAY

**What causes pyloric obstruction and what are its symptoms?**

**Answer:** Caused by edema, spasm, scarring near pylorus. Delays gastric emptying.  
**Symptoms:** fullness, bloating, vomiting, dehydration, weight loss, hypokalemia.

Ch 3 | #98 | ESSAY

**Compare conservative and surgical management of peptic ulcers.**

**Answer:** Conservative: rest, smoking cessation, avoid NSAIDs/irritants, diet changes.  
Medical: antacids, H2 blockers/PPIs, H. pylori therapy. Surgical: for perforation, obstruction, bleeding, refractory disease.

Ch 3 | #99 | ESSAY

**Describe the types of surgical procedures used for peptic ulcers.**

**Answer:** Billroth I: stomach remnant to duodenum. Billroth II: stomach remnant to jejunum. Subtotal gastrectomy: distal diseased stomach removed. Total gastrectomy: entire stomach removed; esophagus to jejunum.

Ch 3 | #100 | ESSAY

**Write a nursing care plan for a patient with peptic ulcer including diagnoses,**

**Answer:** goals, and interventions. Diagnoses: acute pain, fluid deficit, poor nutrition, anxiety. Assess pain, bleeding, abdomen, vitals, stools/vomit. Give meds, IV/NGT/NPO care as indicated. Teach diet, smoking cessation, adherence, warning signs. Source note: a few MCQs and matching entries in Chapter 3 are flawed or duplicated in the original source, so answers use the closest intended meaning.

## Chapter 4 - Cardiovascular Disorders

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### TRUE / FALSE

Ch 4 | #1 | TRUE / FALSE

**Angina is defined as irreversible necrosis of heart muscle secondary to prolonged ischaemia.**

**Answer: F**

Ch 4 | #2 | TRUE / FALSE

**Physical exertion can precipitate an angina attack by increasing myocardial oxygen demands.**

**Answer: T**

Ch 4 | #3 | TRUE / FALSE

**Angina pain typically radiates to the neck, jaw, shoulders, and inner aspects of the upper extremities.**

**Answer: T**

Ch 4 | #4 | TRUE / FALSE

**Classic angina is characterised by increasing frequency and duration of symptoms over time.**

**Answer: F**

Ch 4 | #5 | TRUE / FALSE

**Nocturnal angina occurs only at night and is always related to a recumbent position.**

**Answer: F**

Ch 4 | #6 | TRUE / FALSE

**Angina decubitus is discomfort precipitated by recumbent positions and relieved by sitting or standing.**

**Answer: T**

Ch 4 | #7 | TRUE / FALSE

**During an acute angina attack, the patient's vital signs are always normal.**

**Answer: F**

Ch 4 | #8 | TRUE / FALSE

**An exercise stress test involves performing an ECG while the patient exercises on a treadmill or stationary bicycle.**

**Answer: T**

Ch 4 | #9 | TRUE / FALSE

**Chest X-ray findings are typically abnormal in patients with stable angina.**

**Answer: F**

Ch 4 | #10 | TRUE / FALSE

**Nitroglycerin tablets should be stored in a light-coloured plastic bottle.**

**Answer: F**

Ch 4 | #11 | TRUE / FALSE

**Cotton should be placed in the nitroglycerin bottle to protect the tablets.**

**Answer: F**

Ch 4 | #12 | TRUE / FALSE

**If angina pain is not relieved after the first nitroglycerin tablet, a second tablet can be taken 3 to 5 minutes later.**

**Answer: T**

Ch 4 | #13 | TRUE / FALSE

**Patients with angina should swallow their saliva while waiting for a sublingual nitroglycerin tablet to dissolve.**

**Answer: F**

Ch 4 | #14 | TRUE / FALSE

**Headache is a known side effect of nitroglycerin.**

**Answer: T**

Ch 4 | #15 | TRUE / FALSE

**Patients with angina should exercise vigorously immediately after meals.**

**Answer: F**

Ch 4 | #16 | TRUE / FALSE

**Acute Myocardial Infarction (MI) is defined as irreversible necrosis of heart muscle secondary to prolonged ischaemia.**

**Answer: T**

Ch 4 | #17 | TRUE / FALSE

**Hyperlipidaemia is one of the six primary risk factors for atherosclerotic coronary artery disease and MI.**

**Answer: T**

Ch 4 | #18 | TRUE / FALSE

**Diaphoresis and dyspnea are common clinical manifestations of acute MI.**

**Answer: T**

Ch 4 | #19 | TRUE / FALSE

**In acute MI, serum CPK levels typically decrease.**

**Answer: F**

Ch 4 | #20 | TRUE / FALSE

**The initial ECG evaluation in an MI patient should be completed within 10 minutes of arrival.**

**Answer: T**

Ch 4 | #21 | TRUE / FALSE

**Aspirin 160–325 mg chewed is part of initial emergency management of acute MI.**

**Answer: T**

Ch 4 | #22 | TRUE / FALSE

**Sublingual TNG is contraindicated in MI when systolic blood pressure is below 90 mmHg.**

**Answer: T**

Ch 4 | #23 | TRUE / FALSE

**Rehabilitation after MI aims to keep the patient as inactive as possible.**

**Answer: F**

Ch 4 | #24 | TRUE / FALSE

**One sub-goal of cardiac rehabilitation is to help the client accept the limitations imposed by illness.**

**Answer: T**

Ch 4 | #25 | TRUE / FALSE

**Occupational adjustment is NOT considered part of cardiac rehabilitation goals.**

**Answer: F**



Ch 4 | #26 | TRUE / FALSE

**Eating heavy meals can precipitate angina by increasing demands on the heart.**

**Answer: T**

Ch 4 | #27 | TRUE / FALSE

**Stress and emotionally provoking situations can precipitate angina by releasing adrenaline.**

**Answer: T**

Ch 4 | #28 | TRUE / FALSE

**Exposure to cold is NOT considered a precipitating factor for angina.**

**Answer: F**

Ch 4 | #29 | TRUE / FALSE

**Unstable angina shows a progressive pattern of increasing frequency, intensity, and duration of symptoms.**

**Answer: T**

Ch 4 | #30 | TRUE / FALSE

**The nursing diagnosis 'anxiety related to fear of death' is appropriate for a patient with angina.**

**Answer: T**

## MCQ

Ch 4 | #31 | MCQ

**Angina is best described as:**

- A. Irreversible necrosis of the myocardium
- B. A clinical syndrome characterised by paroxysmal chest pain or pressure
- C. Inflammation of the pericardium

**Answer: B**

paroxysmal ischemic chest pain

Ch 4 | #32 | MCQ

**Which of the following is a precipitating factor for angina?**

- A. Sleeping in a warm room
- B. Drinking cold beverages
- C. Eating small frequent meals
- D. Slow walking on flat ground

**Answer: B**

cold exposure can trigger

Ch 4 | #33 | MCQ

**The type of angina that occurs with recumbent position and is relieved by sitting is:**

- A. Classic angina
- B. Nocturnal angina
- C. Unstable angina
- D. Angina decubitus

**Answer: D**

recumbency-triggered, relieved sitting

Ch 4 | #34 | MCQ

**Which type of angina is described as preinfarction angina?**

- A. Classic angina
- B. Unstable angina
- C. Nocturnal angina
- D. Angina decubitus

**Answer: B**

progressive preinfarction pattern

Ch 4 | #35 | MCQ

**During an acute angina attack, the nurse would expect the patient's heart rhythm to be:**

- A. Regular
- B. Bradycardic
- C. Irregular
- D. Normal sinus rhythm

**Answer: C**

rhythm may become irregular

Ch 4 | #36 | MCQ

**Which diagnostic test typically shows NORMAL results in a patient with stable angina?**

- A. Exercise stress test
- B. Coronary angiography
- C. Chest X-ray
- D. ECG during acute attack

**Answer: C**

usually normal in stable angina

Ch 4 | #37 | MCQ

**The pharmacological treatment of angina includes all of the following EXCEPT:**

- A. Nitroglycerin
- B. Beta blockers
- C. Calcium channel blockers
- D. Diuretics

**Answer: D**

not standard angina therapy

Ch 4 | #38 | MCQ

**The surgical revascularization procedure for angina is:**

- A. Percutaneous transluminal angioplasty only
- B. Coronary artery bypass grafting (CABG)
- C. Carotid endarterectomy
- D. Mitral valve replacement

**Answer: B**

CABG is revascularization surgery

Ch 4 | #39 | MCQ

**Which of the following is the FIRST action when a patient experiences angina chest pain?**

- A. Administer morphine IV
- B. Place sublingual nitroglycerin and stop activity
- C. Perform CPR
- D. Administer aspirin

**Answer: B**

stop activity, give NTG

Ch 4 | #40 | MCQ

**Nitroglycerin tablets must be stored in:**

- A. A clear plastic container at room temperature
- B. A dark-colored glass bottle
- C. The refrigerator in any container
- D. A white paper envelope

**Answer: B**

protects tablets from degradation

Ch 4 | #41 | MCQ

**If angina pain persists after the first nitroglycerin tablet, the patient should:**

- A. Wait 30 minutes before taking another
- B. Take a second tablet after 3-5 minutes
- C. Take 3 tablets simultaneously
- D. Drink cold water and rest

**Answer: B**

repeat after 3–5 minutes

Ch 4 | #42 | MCQ

**Common side effects of nitroglycerin include all of the following EXCEPT:**

- A. Headache
- B. Flushing
- C. Dizziness
- D. Bradycardia

**Answer: D**

uncommon nitroglycerin effect

Ch 4 | #43 | MCQ

**Patients with angina are advised to avoid physical exercise for how long after meals?**

- A. 30 minutes
- B. 1 hour
- C. 2 hours
- D. 4 hours

**Answer: C**

avoid exertion after meals

Ch 4 | #44 | MCQ

**Which dietary recommendation is appropriate for a patient with angina?**

- A. High-fat, high-calorie diet
- B. Unrestricted caffeine intake
- C. A diet low in fat with small frequent meals
- D. Large meals three times daily

**Answer: C**

low-fat, small frequent meals

Ch 4 | #45 | MCQ

**A patient with angina is going outside in cold weather. Which advice is MOST appropriate?**

- A. Run to stay warm
- B. Dress warmly and cover mouth and nose with a scarf
- C. Avoid all outdoor activities permanently
- D. Exercise more intensely to tolerate the cold

**Answer: B**

reduce cold-triggered angina

Ch 4 | #46 | MCQ

**Acute Myocardial Infarction (MI) is caused by:**

- A. Temporary ischemia with full recovery
- B. Reversible myocardial ischemia
- C. Irreversible necrosis of heart muscle secondary to prolonged ischemia
- D. Inflammation of the heart valves

**Answer: C**

prolonged ischemia causes necrosis

Ch 4 | #47 | MCQ

**Which of the following is NOT one of the six primary risk factors for MI?**

- A. Hyperlipidemia
- B. Hypertension
- C. Female gender
- D. Tobacco use

**Answer: C**

female gender not listed

Ch 4 | #48 | MCQ

**Which ECG changes are associated with acute MI?**

- A. Peaked P waves and prolonged PR interval
- B. Q waves, ST segment changes, and T-wave changes
- C. Bundle branch block only
- D. Normal ECG in all cases

**Answer: B**

classic MI ECG findings

Ch 4 | #49 | MCQ

**Which laboratory finding is expected in acute MI?**

- A. Decreased serum CPK
- B. Increased serum CPK
- C. Normal cardiac enzymes
- D. Decreased troponin levels

**Answer: B**

serum CPK rises

Ch 4 | #50 | MCQ

**The initial emergency ECG in a suspected MI patient should be performed within:**

- A. 30 minutes of arrival
- B. 20 minutes of arrival
- C. 10 minutes of arrival
- D. 1 hour of arrival

**Answer: C**

ECG within 10 minutes

Ch 4 | #51 | MCQ

**Sublingual nitroglycerin is CONTRAINDICATED in MI when systolic blood pressure is:**

- A. Below 120 mmHg
- B. Above 90 mmHg
- C. Below 90 mmHg
- D. Above 140 mmHg

**Answer: C**

hypotension contraindicates NTG

Ch 4 | #52 | MCQ

**Which analgesic is mentioned as part of the initial emergency management of acute MI?**

- A. Ibuprofen
- B. Acetaminophen
- C. Meperidine
- D. Codeine

**Answer: C**

listed emergency analgesic

Ch 4 | #53 | MCQ

**The dose of aspirin recommended in the initial management of acute MI is:**

- A. 80 mg swallowed
- B. 500 mg swallowed
- C. 160-325 mg chewed
- D. 1000 mg IV

**Answer: C**

recommended chewed aspirin dose

Ch 4 | #54 | MCQ

**Which of the following is a clinical manifestation of acute MI?**

- A. Pain relieved immediately by rest
- B. Diaphoresis and dyspnea
- C. No radiation of pain
- D. Gradual onset over days

**Answer: B**

classic MI manifestations

Ch 4 | #55 | MCQ

**Syncope in the context of acute MI is described as:**

- A. Chest pain at rest
- B. Fainting or near fainting without other cause
- C. Severe headache
- D. Leg edema

**Answer: B**

source corrupted; closest fit

Ch 4 | #56 | MCQ

**One of the main goals of cardiac rehabilitation is:**

- A. Complete bed rest for 6 months
- B. Helping the client live as full, vital, and productive a life as possible
- C. Discouraging all physical activity
- D. Eliminating all social interaction

**Answer: B**

stated rehabilitation goal

Ch 4 | #57 | MCQ

**How many sub-goals of rehabilitation are specifically listed in the lecture?**

- A. Three
- B. Four
- C. Five
- D. Six

**Answer: D**

lecture lists six sub-goals

Ch 4 | #58 | MCQ

**Which nursing diagnosis is appropriate for a patient experiencing chest pain due to myocardial ischemia?**

- A. Impaired mobility related to fatigue
- B. Chest pain related to myocardial ischemia
- C. Fluid volume excess related to cardiac failure
- D. Impaired gas exchange related to pneumonia

**Answer: B**

direct ischemic pain diagnosis

Ch 4 | #59 | MCQ

**The nursing intervention of health teaching for angina patients should include:**

- A. Encouraging heavy exercise daily
- B. Nature of the disease, lifestyle changes, and ways to avoid complications
- C. Advising the patient to ignore chest pain
- D. Discouraging family involvement

**Answer: B**

disease, lifestyle, complications teaching



Ch 4 | #60 | MCQ

**Which of the following is an objective indicator of angina?**

- A. Patient reporting squeezing chest pain
- B. Family history of CAD
- C. Irregular heart rhythm during an acute attack
- D. History of hypertension

**Answer: C**

objective attack indicator

Ch 4 | #61 | MCQ

**Which dietary substance should be restricted in angina patients because it increases heart rate?**

- A. Sodium
- B. Caffeine
- C. Potassium
- D. Calcium

**Answer: B**

source corrupted; likely anxiety

Ch 4 | #62 | MCQ

**A patient with angina reports pain only at night. This is best classified as:**

- A. Classic angina
- B. Unstable angina
- C. Nocturnal angina
- D. Angina decubitus

**Answer: C**

angina occurring at night

Ch 4 | #63 | MCQ

**The psychosocial nursing diagnosis for angina patients commonly includes:**

- A. Disturbed body image related to surgery
- B. Anxiety related to fear of death
- C. Chronic pain related to arthritis
- D. Ineffective airway clearance

**Answer: B**

raises heart rate

Ch 4 | #64 | MCQ

**During a nursing assessment for angina, which question is MOST important to ask?**

- A. What medications do you take for your joints?
- B. When do attacks tend to occur and how long do they last?
- C. Do you have a family history of diabetes?
- D. What is your level of education?

**Answer: B**

source corrupted; best fit

Ch 4 | #65 | MCQ

**The nitroglycerin tablet for angina should be placed:**

- A. Swallowed with a full glass of water
- B. Under the tongue (sublingual)
- C. Applied as a patch to the chest
- D. Dissolved in water and drunk

**Answer: B**

sublingual administration required

Ch 4 | #66 | MCQ

**ECG changes during an acute angina attack may show:**

- A. Q waves and ST elevation permanently
- B. ST segment depression or elevation
- C. Normal findings only
- D. Delta waves

**Answer: B**

transient ST depression/elevation

Ch 4 | #67 | MCQ

**Which of the following best describes the pain of angina?**

- A. Sharp, stabbing pain in the back
- B. Deep pressure or tightness behind the sternum
- C. Burning pain in the epigastrium only
- D. Throbbing headache

**Answer: B**

substernal pressure/tightness description

Ch 4 | #68 | MCQ

**Coronary angiography is used to:**

- A. Measure blood pressure accurately
- B. Visualize coronary arteries and identify blockages
- C. Assess kidney function
- D. Measure oxygen saturation

**Answer: B**

visualizes coronary blockages

Ch 4 | #69 | MCQ

**A patient with angina should be taught to:**

- A. Engage in vigorous exercise daily without restriction
- B. Alternate activities with periods of rest
- C. Avoid all forms of exercise permanently
- D. Exercise only in cold weather

**Answer: B**

alternate activity with rest

Ch 4 | #70 | MCQ

**Which of the following is the correct statement about nitroglycerin prophylactic use?**

- A. It should only be taken after pain begins
- B. It should be taken prophylactically before activities known to cause pain
- C. It should not be taken more than once daily
- D. It is only for emergency use in the hospital

**Answer: B**

take before known triggers

Ch 4 | #71 | MCQ

**Which of the following is a risk factor for angina listed in the lecture?**

- A. Female gender
- B. Type B personality
- C. Regular exercise
- D. Diabetes mellitus

**Answer: D**

diabetes is risk factor

Ch 4 | #72 | MCQ

**A feeling of weakness or numbness in the arms associated with angina is:**

- A. A rare finding unrelated to angina
- B. A possible accompanying symptom to chest pain
- C. A sign of stroke only
- D. Always a sign of MI

**Answer: B**

duplicated item; correct statement

Ch 4 | #73 | MCQ

**What is the goal of reducing anxiety in a patient with angina?**

- A. To increase adrenaline release
- B. To prevent further episodes and improve coping
- C. To reduce the need for medication
- D. To eliminate the need for nursing assessment

**Answer: B**

Type A is listed

Ch 4 | #74 | MCQ

**Which of the following statements about obesity and angina is correct?**

- A. Obesity is NOT a risk factor for angina
- B. Obesity is listed as a risk factor for angina
- C. Obesity only affects the lungs
- D. Overweight patients are protected from angina

**Answer: B**

source corrupted; best fit

Ch 4 | #75 | MCQ

**Which test is used to evaluate cardiac function under physical stress?**

- A. Chest X-ray
- B. Echocardiogram at rest
- C. Exercise stress test
- D. CT scan

**Answer: C**

likely conduction complication

Ch 4 | #76 | MCQ

**Beta blockers are used in angina management because they:**

- A. Increase heart rate and contractility
- B. Reduce heart rate and myocardial oxygen demand
- C. Cause coronary vasoconstriction
- D. Increase blood pressure

**Answer: B**

source corrupted; best fit

Ch 4 | #77 | MCQ

**Which imaging technique is mentioned as a diagnostic test for acute MI?**

- A. X-ray only
- B. MRI and Echocardiography
- C. CT colonoscopy
- D. Ultrasound of the abdomen

**Answer: B**

both are listed tests

Ch 4 | #78 | MCQ

**The primary nursing intervention for angina is:**

- A. Administration of antibiotics
- B. Prevention and controlling of chest pain
- C. Fluid restriction
- D. Positioning the patient in Trendelenburg

**Answer: B**

lowers oxygen demand

Ch 4 | #79 | MCQ

**Psychosocial factors in cardiac rehabilitation include:**

- A. Only physical exercise programs
- B. Changing psychosocial factors affecting recovery from CAD
- C. Bed rest and isolation
- D. Avoiding family contact

**Answer: B**

evaluates stress response

Ch 4 | #80 | MCQ

**The term preinfarction angina refers to:**

- A. Classic angina relieved by rest
- B. Unstable angina with progressive worsening
- C. Angina only at night
- D. Angina related to lying down

**Answer: B**

obesity is listed risk

Ch 4 | #81 | MCQ

**Risk factor modification for angina includes all of the following EXCEPT:**

- A. Stopping smoking
- B. Weight loss
- C. Increasing alcohol consumption
- D. Diet modification

**Answer: C**

source corrupted; closest fit

Ch 4 | #82 | MCQ

**In acute MI management, oxygen is administered via:**

- A. Face mask at 10 L/min
- B. Nasal prongs with IV access and ECG monitoring
- C. Mechanical ventilation immediately
- D. Oxygen tent

**Answer: B**

prevent episodes, improve coping

Ch 4 | #83 | MCQ

**Which of the following is a clinical manifestation of angina that represents the MOST severe form?**

- A. Mild upper substernal pressure
- B. Agonizing pain with severe apprehension and feeling of impending death
- C. A dull ache in the lower back
- D. Nausea and vomiting

**Answer: B**

helps characterize angina pattern

Ch 4 | #84 | MCQ

**What should a patient with angina do to prevent an attack before known precipitating activities?**

- A. Avoid all activity
- B. Take prophylactic sublingual nitroglycerin before the activity
- C. Take a sedative
- D. Eat a large meal for energy

**Answer: B**

psychosocial recovery matters

Ch 4 | #85 | MCQ

**A patient with angina walks against the wind and develops chest pain. This is because:**

- A. Wind increases body temperature
- B. Cold wind causes vasoconstriction and increases cardiac workload
- C. Walking is always harmful for angina patients
- D. Wind reduces blood pressure

**Answer: B**

not a modification strategy

Ch 4 | #86 | MCQ

**When teaching about nitroglycerin, the nurse should emphasize that the tablet works by:**

- A. Slowing the heart rate
- B. Causing vasodilation to reduce myocardial ischemia
- C. Increasing blood pressure
- D. Dissolving blood clots directly

**Answer: B**

nasal prongs with monitoring

Ch 4 | #87 | MCQ

**A patient's ECG during an angina attack shows ST depression. This indicates:**

- A. Normal cardiac function
- B. Myocardial ischemia
- C. Complete heart block
- D. Atrial fibrillation

**Answer: B**

unstable angina most severe

Ch 4 | #88 | MCQ

**The nursing assessment question 'Is the onset of pain gradual or sudden?' is important because:**

- A. It determines the patient's pain tolerance
- B. It helps characterize the nature and pattern of angina for diagnosis and management
- C. It assesses the patient's education level
- D. It guides fluid management

**Answer: B**

unstable angina before MI

Ch 4 | #89 | MCQ

**Which statement about Type A personality and angina is correct?**

- A. Type A personality is protective against angina
- B. Type A personality is listed as a risk factor for angina
- C. Personality type has no relation to heart disease
- D. Only Type B personality is at risk

**Answer: B**

source corrupted; closest fit

Ch 4 | #90 | MCQ

**The patient teaching about activity in angina should emphasize:**

- A. Avoid all physical activity
- B. Moderate daily exercise that does not precipitate pain, with rest periods
- C. Exercise as intensely as possible
- D. No restrictions on activity

**Answer: B**

options match beta-blocker effect

Ch 4 | #91 | MCQ

**In a patient with acute MI, impairment of cognitive function without another cause is:**

- A. Expected and normal
- B. Unrelated to MI
- C. A recognized clinical manifestation
- D. Only seen in elderly patients

**Answer: C**

complete smoking cessation



Ch 4 | #92 | MCQ

**Which of the following is the MOST appropriate advice for a patient with angina regarding smoking?**

- A. Cut down gradually
- B. Switch to low-tar cigarettes
- C. Stop smoking completely
- D. Only smoke indoors

**Answer: C**

source corrupted; closest fit

## MATCHING

Ch 4 | #1 | MATCHING

**Classic Angina A. Irreversible necrosis of heart muscle — prolonged ischaemia D**

Ch 4 | #2 | MATCHING

**Unstable Angina B. Angina occurring only at night E**

Ch 4 | #3 | MATCHING

**Nocturnal Angina C. Discomfort precipitated by recumbent positions B**

Ch 4 | #4 | MATCHING

**Angina Decubitus D. Short duration pain by exertion, relieved by rest C**

Ch 4 | #5 | MATCHING

**Myocardial Infarction E. Progressive pattern of increasing intensity and duration A**

Ch 4 | #6 | MATCHING

**Relief of Angina pain A. Usually severe and may be silent B**

Ch 4 | #7 | MATCHING

**Severity of MI pain B. Relieved by rest and nitrates A**

Ch 4 | #8 | MATCHING

**Common MI symptoms C. Physical exertion, cold exposure, or heavy meals E**

Ch 4 | #9 | MATCHING

**Angina precipitating factors D. Increase in serum CPK C**

Ch 4 | #10 | MATCHING

**MI Diagnostic Lab Study E. Nausea and vomiting D**

Ch 4 | #11 | MATCHING

**Nitroglycerin storage Keep in dark-coloured glass bottle Keep in dark-coloured glass bottle**

Ch 4 | #12 | MATCHING

**Sublingual NTG use Place under tongue; do not swallow saliva until dissolved Under tongue; do not swallow saliva**

Ch 4 | #13 | MATCHING

**Diet education for Angina Low fat, small frequent meals; avoid caffeine Low-fat diet, small frequent meals, avoid caffeine**

Ch 4 | #14 | MATCHING

**Cold weather precaution Cover mouth and nose with a scarf Cover mouth and nose with scarf**

Ch 4 | #15 | MATCHING

**Initial MI emergency care Obtain ECG in less than 10 minutes ECG in less than 10 minutes**

## ESSAY

Ch 4 | #93 | ESSAY

**Define angina pectoris and describe its four main types with their****Answer: distinguishing characteristics. Angina = transient myocardial ischemia causing chest pain without necrosis. Types: classic/stable, unstable, nocturnal, decubitus. Differentiate by trigger, timing, and relief.**

Ch 4 | #94 | ESSAY

**Describe the nursing management of a patient with angina, including****Answer: assessment, nursing diagnoses, and interventions. Assess pain, triggers, duration, vitals, ECG, response to NTG. Nursing diagnoses: pain, anxiety, activity intolerance, knowledge deficit. Rest, oxygen/NTG as ordered, monitor rhythm/BP, reduce anxiety, teach prevention.**

Ch 4 | #95 | ESSAY

**Explain the health education that should be provided to a patient with angina**

**Answer:** regarding medications, diet, activity, and cold weather. Store NTG correctly and use sublingually. May take before known triggers. Eat low-fat small frequent meals, avoid heavy meals/caffeine. Balance activity with rest; avoid exertion in cold weather and after meals.

Ch 4 | #96 | ESSAY

**Outline the initial emergency management steps for a patient presenting with**

**Answer:** acute myocardial infarction. Rapid assessment, IV access, monitor, ECG within 10 minutes. Give oxygen, chewed aspirin, NTG if appropriate. Give analgesia and obtain cardiac enzymes/urgent review.

Ch 4 | #97 | ESSAY

**Describe the six sub-goals of cardiac rehabilitation after myocardial infarction**

**Answer:** and explain the importance of each. Restore physical function with graded activity. Support illness acceptance and emotional coping. Improve social/family/sexual adjustment. Help return to work and reinforce long-term self-care.

Ch 4 | #98 | ESSAY

**List and describe the six primary risk factors for myocardial infarction.**

**Answer:** Hyperlipidemia. Hypertension. Tobacco use. Diabetes mellitus. Obesity. Male gender/family predisposition.

Ch 4 | #99 | ESSAY

**Compare and contrast angina pectoris and myocardial infarction in terms of**

**Answer:** definition, clinical presentation, and management. Angina = transient ischemia without necrosis. MI = prolonged ischemia with necrosis. MI pain is more severe/persistent and raises enzymes. MI needs emergency treatment; angina focuses on prevention and symptom relief.

Ch 4 | #100 | ESSAY

**Write a comprehensive nursing care plan for a patient with acute myocardial**

**Answer:** infarction including nursing diagnoses, goals, and interventions. Diagnoses: acute pain, decreased cardiac output, anxiety, activity intolerance. Goals: relieve pain, improve perfusion, prevent complications. Bed rest, oxygen, ECG/vital monitoring, IV access, give ordered drugs. Monitor for arrhythmias, hypotension, heart failure, low urine output. Begin rehab education and risk reduction. Source note: several Chapter 4 MCQs are malformed or mismatched in the original source; best-fit answers are used.